

1st week Sunday case scenario

CASE 1

Mrs saz a 46-year-old shop assistant who is married with four children. She is referred by her GP to the Gynaecology clinic; she was investigated for tiredness and lethargy and was diagnosed with anaemia. Her GP suspects that her periods might be to blame. Her cycles are 7/29, and there is no intermenstrual bleeding. She uses condoms for contraception.

1.1. Visit the NICE.org.uk website, and read the Quick Reference guidance on Heavy Menstrual Bleeding (HMB) (<http://www.nice.org.uk/guidance/CG44/quickrefguide/pdf>)

1.2. Suggest questions that you could ask Mrs Layla to quantify her menstrual loss.

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1.3. You decide that the history does suggest heavy periods. What features of the physical examination, other than internal pelvic and speculum examination, might be helpful in your differential diagnosis?

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You examine the patient and find her generally well, with a normal-size mobile uterus and no adnexal masses.

1.4.What is the most likely diagnosis and what features of history and examination support your view?

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1.5.Suggest first line investigations, and discuss the rationale for your recommendation:

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1.6.Outline the management options for this patient, starting with the simplest, least invasive treatments first.

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1.7.Explain to the patient the advantages and disadvantages of each of your suggested treatments.

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1.8.Describe the mechanism of action in two of the commonly used non-hormonal therapeutics in the management of heavy menstrual blood loss.

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1.9 What operative interventions are available if medical management fails, and what are their commonest complications?

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